

Nursing Recording and reporting

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To embrace students with knowledge, skills & positive attitude toward Nursing Recording and reporting.

OBJECTIVE

TEXTSTUDIO

At the end of this lecture, students will be able to:

Outlines

- Define key terms
- Enumerate characteristics of good recording and reporting
- List purposes of recording and reporting
- Explain types of records
- Enumerate principles of record writing
- Discuss methods of recording
- Discuss types of reporting
- List role of administrator

Introduction

Nursing documentation is a vital component of safe, ethical and effective nursing practice, regardless of the context of practice or whether the documentation is paper-based or electronic. This document is intended to provide registered nurses (RNs) with guidelines for professional accountability in documentation and to describe the expectant for nursing documentation in all practice settings, regardless of the method or storage of that documentation. The main aim of the document is to assist the registered nurse to meet their standards of practice related to documentation.

Definition of documentation

Documentation is the permanent recording of information properly identified as to time, place, circumstances and attribution.

Documentation is the written, legal record of all pertinent interactions with the client-assessing, diagnosing, planning, implementing and evaluating.

Definitions.

Recording:

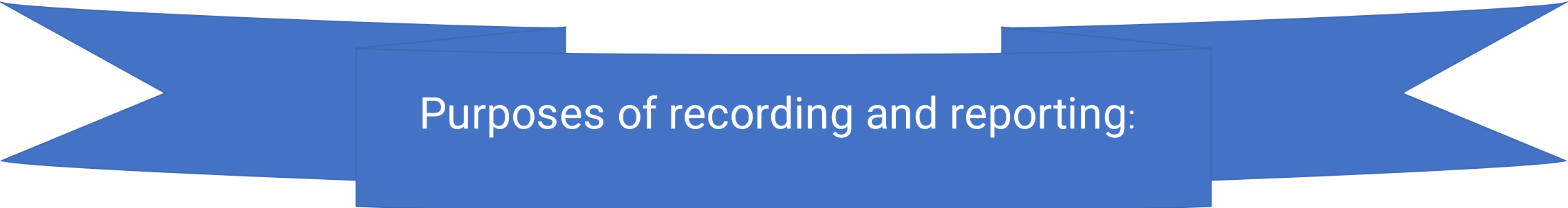
To write (something) down so that it can be used or seen again in the future; to produce a record of (something).

Reporting:

It is oral, written or computerized based communication intended to convey information.

Characteristics of good Recording and Reporting:

- 1-Accuracy: Information should be correct to prevent serious mistakes.
- 2-Conciseness: Use a few words as possible to give the necessary information.
- 3-Thoroughness: Even a concise record or report must contain complete information.
- 4-Up to date: Recording should be done on time.
- 5-Organization: Communicate all the information in a logical format or order.
- 6-Confidentiality: The information should be confidential.
- 7-Objectivity: Presentation of facts not personal feelings, to give true picture.



Purposes of recording and reporting:

1. Decision Making.
2. Communication
4. Legal accountability.
5. Education
6. Assessment.
7. Planning.
8. Research.
10. Historical document.
11. Quality assurance.
12. Vital statistics.
13. Health service planning.
14. Diagnostic and the therapeutic orders.
15. Accrediting and licensing.

Types of records:

1. Ward records
2. Nurses' records.
3. Education records.
4. Administrative records.



1. Ward records

1. Patients clinical record:

It includes outpatient records and inpatient records.

2. Doctor order sheet:

Doctors' order regarding treatments, medications, investigation, and diet may be written on separate sheets.

3. Reports of laboratory examination:

Normal values included in this for evaluating abnormalities.

Cont.,

4. Diet sheet:

It includes type of diet depends on the patient's diagnosis. E.g., for diabetic patient sugar free diet.

5. Consent form for operations and anesthesia:

Before going to do any treatment or surgery consent to be taken from the patient or any responsible person from the patient side.

6. Intake and output chart:

Intake of the patient includes IV fluids, oral fluids, Ryle tube feedings, gastrostomy tube feedings. I/O chart maintained in clients with critical illness, diarrhea, diuretics, after surgery.

Cont.,

7. Reports of physiotherapy, occupational therapy

8. Kardexes, SBAR, ISBAR.

9. Instruction Book:

The doctors give special instant instructions when any incident happens suddenly during rounds.

10. Admission and discharge record:

Record the name, age, sex, address of the client, date and time of the admission, diagnosis of the client. It gives the record of total number of admissions per day.

Cont.,

11. Census Record:

It includes the total number of admissions, discharges, transfer-ins, transfer-outs, absconding and deaths of the client.

12. Call Book:

It includes the name of the doctor, date, time and purpose of the call-in emergency situations.

13. Complaint book:

It consists of any repairs in the ward like machinery, electricity and water supply.

Cont.,

14. Movement register:

It gives the information regarding the availability of doctor and nurse concerned. E.g. place where they are available at that particular time.

15. Indent book:

The total number of linens, medicines, IV fluids, cotton, and gauze is indented for the patient care.

16. Drug's maintenance register:

The prescription and supply of drugs generates a variety of records including pharmacy stock ordering, dispensing records, request for drugs from stores, drug administration records, prescription for individual patients, the receipt and issue of all drugs should be recorded.

2-Nurses records:

- The office of the chief nurse will generate records of the type found in the office with an executive or administrative function: correspondence, reports, minute of meetings.

3-Educational records:

- The officers, boards and committee of medical and nursing schools will produce their own records, minutes, correspondence, reports and so on.

4-Administrative records in nursing superintendent's office:

Hospitals also requires records relating to finance, personnel, building, accommodation, stores and other such services, although they will be little different from those used in non-medical organizations of equivalent size.

1. Philosophy, purposes and curriculum.
2. Course, content and course plan record for each subject.
3. Rotation plan for duties of staff nurses.
4. Record of committee.
5. Record of academic requirement.
6. Stock register.
7. Annual reports.
8. Affiliation records.
9. Written policies.
10. Statement of budget proposal and allotments.
11. Copy of brochure.

Principles of record writing:

1. Legal prudence:

Since the clinical record is a legal document and gives legal protection to the nurse, other health care professional of the institution, it is essential that they should be written clearly, accurately and confidentiality maintained.

2. Legibility:

Writing must be clear and easily readable by others.

3. Organizations:

Documentations on data collection should be organized in a local pattern, as the statement is more easily read.

Cont.

4. All entries should be signed by the individual who writes them.

5. Records are written continuously:

With no blank spaces. If any space is left out, it should be crossed out, dated and signed.

6. Date and Sign:

When recording medications and treatments, note exact time and date on which they are carried out.

7. Accuracy:

Each page of the record should be properly identified with the name, age, I.P. No, Bed No, ward, date etc.

Cont.

9. Sequence and timeliness:

Documentation on is the timely manner can help to avoid errors.

10. Completeness:

Record should be truthful and complete.

11. Correct spelling:

check the dictionary and use the correct spelling.

Methods of recording:

1. Narrative charting
2. Source -oriented charting
3. Problem - oriented charting
4. PIE charting
5. Focus charting
6. Charting by exception
7. Graphic sheets and flow sheets:

1. Narrative charting:

It is a traditional method for recording nursing care provided. It is a story like format to document information specific to client conditions and nursing care.

2. Source -oriented charting:

Descriptive recording done by each member of health care team on separated parts. One of the most prominent features of this problem-orientated method of documentation is the structured way in which narrative progress notes are written by all health-care team members, using the SOAP, SOAPIE OR SOAPIER format.

CONT.

Subjective: the client's observation.

Objective: the care provider's observations.

Assessment: the care providers understanding of the problem.

Plans Goals: The plan outlines the goals and interventions intended to meet the patient's needs.

Intervention: action, advice intervention when an intervention was identified and changed to meet client's needs. Intervention refers to the actions performed by the nurse to treat or manage the patient's problem.

Evaluation: how outcomes of care are evaluated.

Revision: when changes to the original problem come from revised.

3. Problem - oriented charting:

- This is a method of documentation that places emphasis on client's problem. Here, each member of health team contributes to a single list of identified client problems.
- The **nursing process** forms the basis for the POMR method of documenting client's problems listed the advantages of this method of documenting are as follows:
 1. Gives emphasis to client's perceptions of their problems.
 - 2 Requires continuous evaluation and revision of the care plan.
 3. Provides greater continuity of care among health-care team members.
 - 4 Increases efficiency in gathering data.
 5. Reinforces use of the nursing process.

4. PIE charting:

- The key components of the system are assessment flow sheets and nurses progress notes with plan of care. The PIE notes are numbered according to the client problem. Resolved problems are dropped from daily documentation after the RNs review.

5. Focus charting:

- Focus charting is the method of identifying and organizing the narrative documentation of client's concerns. This method of documentation consists of notes that includes data, both subjective and objective; action or nursing interventions; and response of the client. The notes are structured according to clients' concerns.

6. Charting by exception (CBE) :

Is a charting method that requires nurse to record only deviation from established norms.

7. Graphic sheets and flow sheets:

Health care record entries should reflect the most recent assessment, as they are done, to ensure treatment decisions are based on accurate information. If documenting on a flow sheet or checklist, check marks may be used as long as it is clear who performed the assessment or intervention. The meaning of check mark or symbol used must be identified in the practice setting policy.

Types of reporting:

- Reporting is the **verbal** or **written** communication of data regarding the client health status needs, treatments, outcomes and responses.
- Reporting facilitates clinical decision making, continuity of care and co-ordination among health team members.

The reports used in hospital setting usually are:

1. Change - of - shift reports
2. Transfer reports.
3. Incident reports.

Role of administrator:

- The nurse administrator should see that everybody is following common guidelines for recording information:
 - Information recorded is true and complete.
 - Entries should be legible and written in ink.
 - Only facts should be recorded.
 - Entries should be brief, accurate, legible and correctly spelt.
 - If item error is made while written, the nurse should not erase or overwrite, instead draw a single line over it and sign it. Then note it down correctly.
 - Do not leave blank space in a note.
 - Should write in chronological order of date and time.
 - Each page of record should be properly identified with identification data.

Summary

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A person in a dark suit and tie is shown from the chest up, holding a glowing, rectangular box with the text "ANY QUESTIONS?" in white, bold, sans-serif capital letters. The box has a blue and white border and a slight 3D effect. The background is a blurred office setting with a desk, a pen, and a blue mug. Overlaid on the image are several digital icons: a puzzle piece, a globe, a speech bubble, a padlock, and gears, all connected by thin, glowing blue lines. The overall color scheme is dominated by blues and greys, with the glowing box providing a bright focal point.

**ANY
QUESTIONS?**

